# Case Taking

Clinical Postings | 3rd Year MBBS

Hi Sammy!
History taking = 80%
of the diagnosis!
Master this art!

Case Taking | Clinical Postings | 3rd Year | MBBS

## What's Inside


- Case Sheet Format & History Taking Technique p.2
- Chief Complaints, HOPI & Past History p.3
- Personal, Family History & Systemic Review p.4
- Exam Strategy & High Yield p.5

# Case Sheet Format + Technique

A good history =
half the diagnosis!
Follow the format!

**GOLDEN RULE OF HISTORY TAKING**

*Listen to the patient — they are telling you the diagnosis. 80% of diagnoses come from history alone!*

## Standard Case Sheet Format

| | | |
|---|---|---|
| 1 | Biodata | Name, Age, Sex, Religion, Occupation, Address, DOA, DOE |
| 2 | C/C | Chief Complaints — in patient's own words with duration |
| 3 | HOPI | History of Present Illness — detailed chronological narrative |
| 4 | Past Hx | Past medical, surgical, drug history, allergies |
| 5 | Personal Hx | Diet, sleep, appetite, bowel/bladder, habits, addictions |
| 6 | Family Hx | Similar illness, hereditary diseases, TB/DM/HTN contacts |
| 7 | Menstrual Hx | LMP, cycle regularity, obstetric history (females) |
| 8 | Rx History | Current medications, previous treatments, compliance |

## History Taking Technique — Do's + Don'ts

**DO's**

- Introduce yourself & explain purpose
- Stand on RIGHT side of patient
- Use open-ended questions first
- Maintain eye contact & empathy
- Record in chronological order

**DON'Ts**

- Don't use medical jargon with patient
- Don't interrupt the patient
- Don't ask leading questions
- Don't skip any section of history
- Don't write diagnosis before exam

HOPI = the HEART of case taking! Spend max time here!

## Chief Complaints (C/C)

- Record in patient's OWN words — not medical terms!
- List in chronological order with DURATION (e.g., fever x 5 days)
- Maximum 3-4 chief complaints (most important first)
- Each complaint = separate line with onset & duration

## History of Present Illness (HOPI)

- Detailed chronological narrative of EACH chief complaint
- For EACH symptom, describe using SOCRATES mnemonic:

| | |
|---|---|
| S — Site: | Where is the symptom? |
| O — Onset: | When did it start? Sudden/gradual? |
| C — Character: | Nature of pain (sharp, dull, burning)? |
| R — Radiation: | Does it spread anywhere? |
| A — Associations: | Other symptoms occurring with it? |
| T — Timing: | Constant/intermittent? Duration? |

★ **SOCRATES = Pain History!**
Site, Onset, Character, Radiation, Associations, Timing, Exacerbating, Severity

## Past History — What to Ask

| Category | Details to Ask |
|---|---|
| Medical History | DM, HTN, TB, asthma, epilepsy, thyroid, heart disease |
| Surgical History | Previous surgeries — type, year, complications |
| Drug History | Current medications, dose, duration, compliance |
| Allergy History | Drug allergies, food allergies — type of reaction |
| Blood Transfusion | History of transfusion, any reaction |
| Hospitalization | Previous admissions — reason, duration, outcome |

# Personal, Family Hx + Systemic Review

Don't skip personal & family history! They reveal hidden clues!

## Personal History

| Aspect | What to Record |
|---|---|
| **Diet** | Vegetarian/non-veg, appetite, any recent change |
| **Sleep** | Pattern, insomnia, excessive sleep, disturbances |
| **Bowel/Bladder** | Frequency, consistency, blood/mucus, burning micturition |
| **Habits** | Smoking (pack-years), alcohol (units), tobacco chewing |
| **Occupation** | Type of work, exposure to dust/chemicals/radiation |
| **Marital Status** | Married/single, sexual history if relevant |

## Family History

- Similar illness in family members (genetic/hereditary clue)
- DM, HTN, heart disease, cancer, TB — in first-degree relatives
- Consanguinity (important in autosomal recessive disorders)
- Cause of death of parents/siblings if applicable

## Review of Systems (Systemic Enquiry)

| System | Key Symptoms to Ask |
|---|---|
| **CVS** | Chest pain, palpitations, dyspnea, orthopnea, edema feet |
| **Respiratory** | Cough, sputum, hemoptysis, wheeze, dyspnea |
| **GIT** | Nausea, vomiting, pain abdomen, diarrhea, constipation, jaundice |
| **CNS** | Headache, dizziness, seizures, weakness, numbness, vision change |
| **Musculoskeletal** | Joint pain, swelling, stiffness, restricted movement |
| **Genitourinary** | Dysuria, frequency, hematuria, discharge, menstrual issues |
| **Endocrine** | Weight change, heat/cold intolerance, polyuria, polydipsia |

## Common Mistakes to Avoid!

- Writing diagnosis in chief complaints (e.g., 'Typhoid' instead of 'fever x 7 days')
- Not recording duration with each complaint
- Skipping personal/family history (examiners love to grill you here!)
- Using medical jargon instead of patient's own words for C/C
- Not doing systemic review (misses other system involvement)

# Exam Strategy + High Yield

Case taking = your
clinical exam foundation!
You've GOT this Sammy!

## Most Repeated LAQs (10–12 marks)

1. Write the format of a complete case sheet with all components
2. Describe the technique of history taking with do's and don'ts
3. Take a detailed history of a patient presenting with chest pain
4. Take history of a patient with fever — include all relevant sections

## Most Repeated SAQs (5–6 marks)

1. SOCRATES mnemonic — explain with example
2. Components of personal history and their importance
3. What is review of systems? Name symptoms for each system
4. Difference between chief complaint and HOPI
5. Why is family history important? Give examples
6. Common mistakes in history taking by medical students

## One-Liner Associations (MCQ/Viva Gold)

| Question | Answer |
|---|---|
| Most important part of diagnosis | History (80% of diagnoses) |
| C/C should be recorded in | Patient's own words with duration |
| SOCRATES stands for | Site, Onset, Character, Radiation… |
| Pack-years formula | Packs/day x years of smoking |
| Consanguinity is important for | Autosomal recessive disorders |
| HOPI should be written in | Chronological narrative format |
| Stand on which side of patient? | Right side |
| Open vs Closed questions | Open first, then narrow down |
| Menstrual Hx must include | LMP, cycle length, regularity, flow |
| Drug allergy must specify | Drug name + type of reaction |

♥ Every patient has a story, Sammy. Your job is to listen!
Good history = great doctor. And you're becoming a great one!